Self-therapy Guidebook:

COMPLEX POST-TRAUMATIC STRESS DISORDER (CPTSD)

FIRST EDITION

Greylight Psychology

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PLEASE NOTE:

The material within this resource is not intended as a replacement for one-on-one sessions with a qualified mental health professional.

If you are struggling with genuine mental health challenges, it is advised that you are properly assessed and provided with a relevant, evidence-based treatment plan by your attending practitioner.

If you are at risk of self-harm or suicide, please contact the emergency supports local to you, or present to your local emergency department.

CONTENT WARNING:

This guidebook contains references to various types of trauma - if you feel this content may be too triggering, please do not proceed further.

Firstly, welcome!

If you're reading this, you've likely decided to make a significant change in your life, and more to the point, you've chosen to experiment with applying the techniques yourself. This is by no means an easy task, and you will likely encounter additional and unique challenges compared to those working alongside a professional, but you'll also encounter unique and exciting opportunities on this guided journey towards recovery and a greater degree of purpose and happiness.

It must be said that self-therapy is not intended to be a complete replacement of traditional face-to-face therapy with a qualified and experienced professional. Instead, the goal of this ebook is to provide you with an opportunity to experiment with learning and applying the skills taught in evidence-based settings to your own unique situation, and to provide some relief to our overburdened mental health system. More importantly, it seeks to provide relief to you, who might be struggling to find any psychologists local to you with reasonable enough availability.

A few notes on recovery

In psychology, recovery is a dynamic word that encapsulates the ebb and flow of progress through therapy. It is a 'moving' word, in that there are periods of growth and periods of relapse, but regardless, the journey towards a significant remission continues. Is there a point at which someone is considered to have 'recovered'? Possibly, but psychologists generally don't view recovery as a singular point or a threshold that is permanently crossed, never to be stepped back behind again.

As with most issues in life, problems we thought were dealt with can resurface, and the disappointment or shame after reencountering an issue we thought we 'cured' can drag us down far quicker and deeper than necessary. This is not to say that we must live in fear of past challenges reemerging when we're tired or vulnerable, but that we should remain prepared and always within arm's reach of our tools and management strategies.

Given this, recovery is about 'two steps forward, and one step back', and it's vital that we make room for the human element on this journey. We're going to make mistakes, and that's okay. We're going to rebel against what we know we should do, and that's okay too. Ultimately, recovery is about getting up after each fall, dusting ourselves off and soldiering forwards towards what we know we deserve, Human beings, and our brains more specifically, are wholly resistant to change, but the task is not impossible – far from it. With enough will power, discipline and grit, we can start to see a healthier, more idealised quality of life take shape, and this is one of the most beautiful experiences life on this planet has to offer.

A typical course of therapy

A standard course of face-to-face therapy can last anywhere between 10 to 20 sessions, depending on the pace of treatment, client characteristics and the severity of symptoms. The structure of this ebook is divided into Modules, with each Module accounting for approximately 2 to 3 sessions in conventional therapy. While you don't have to space them out fortnightly, you are welcome to do so. You are also free to move as quickly, or as deliberately, as you see fit, and if you have some experience with therapy for generalised anxiety, you may wish to skip straight to the content applicable to your needs and goals.

What is self-therapy?

As the name suggests, self-therapy is about you guiding yourself through the recovery journey. It takes the core principles, psychoeducation and strategies from traditional in-room therapy, and places them at your disposal, while guiding you steadily so as to avoid moving too quickly.

It differs from self-help in that the information in this ebook is rigourously validated and drawn from a large variety of scientific studies, and seeks to emulate the pace and 'feel' of a guided therapeutic course. It is practical, but also clinical, and will focus largely on what has demonstrated to work at reducing symptoms as opposed to what makes you feel 'warm and fuzzy' in the short-term. This is not to say that it's 'superior' to self-help – it's just a different approach, and each individual will likely develop a preference for one over the other.

Contact us!

If you have any questions regarding the material, you can always contact us at our instagram page via direct message: ***@greylight_psychology*** or via our email address: ***enquiries@greylightpsychology.com.au***

We wish you the very best, and truly hope that you find great value in the pages to follow.

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Module 1.1 - Psychoeducation

WHAT IS TRAUMA?

Overview Trauma

Trauma encompasses a spectrum of distressing experiences that overwhelm an individual's ability to cope, leaving lasting psychological, emotional, and sometimes physical scars. It can arise from various sources, including but not limited to, physical or sexual abuse, neglect, accidents, natural disasters, war, and interpersonal violence. The impact of trauma can manifest in diverse ways, affecting every aspect of an individual's life. Common manifestations of trauma include intrusive memories or flashbacks of the traumatic event, avoidance of reminders of the trauma, heightened arousal and reactivity, difficulty regulating emotions, and altered beliefs about oneself, others, and the world. Additionally, individuals may experience disruptions in their relationships, work or academic performance, sleep patterns, and overall sense of well-being. Trauma can profoundly shape one's worldview, influencing how they perceive themselves, others, and the world around them. It's essential to recognize the varied manifestations of trauma and provide compassionate support and resources for individuals navigating the complex aftermath of traumatic experiences.

Trauma and the Brain

Trauma can have profound and enduring effects on the structure and function of the brain and nervous system, leading to alterations in neural circuitry, neurotransmitter systems, and stress response mechanisms. These changes contribute to a range of cognitive, emotional, and physiological symptoms commonly

observed in individuals who have experienced trauma.

Amygdala and Fear Response: The amygdala, a key structure involved in processing emotions, particularly fear, becomes hyperactive in response to trauma. This heightened activity can lead to exaggerated fear responses and hypervigilance, contributing to feelings of anxiety and a heightened startle response.

Hippocampus and Memory: The hippocampus, responsible for memory formation and consolidation, can be adversely affected by trauma. Chronic stress and trauma-related neurochemical changes may impair hippocampal function, leading to difficulties in memory retrieval and consolidation. This can manifest as fragmented memories or difficulties in recalling specific details of traumatic events.

Prefrontal Cortex and Executive Functioning: The prefrontal cortex, involved in higher-order cognitive functions such as decision-making, impulse control, and emotion regulation, may exhibit reduced activity and connectivity following trauma. This can result in difficulties in regulating emotions, controlling impulses, and making reasoned decisions, contributing to symptoms of emotional dysregulation and impulsivity.

Hypothalamic-Pituitary-Adrenal (HPA) Axis Dysregulation: Trauma can dysregulate the HPA axis, a key neuroendocrine system involved in the body's stress response. Chronic exposure to trauma-related stressors can lead to alterations in cortisol levels, disrupting the body's ability to regulate stress effectively. This dysregulation may contribute to symptoms such as sleep disturbances, fatigue, and heightened arousal.

Neurotransmitter Imbalance: Trauma can disrupt the balance of neurotransmitters, such as serotonin, dopamine, and norepinephrine, which play critical roles in mood regulation, reward processing, and arousal. Imbalances in these neurotransmitter systems can contribute to symptoms of depression, anxiety, and mood instability commonly observed in individuals with trauma-related disorders.

Neuroplasticity and Structural Changes: The brain exhibits remarkable neuroplasticity, the ability to reorganize and adapt in response to experience. However, chronic exposure to trauma can lead to maladaptive changes in brain structure and function. These structural alterations may include reductions in gray matter volume, particularly in regions implicated in emotion regulation and stress response, such as the prefrontal cortex and hippocampus.

Overall, trauma-induced changes in the brain and nervous system underscore the complex interplay between biological, psychological, and environmental factors in shaping individuals' responses to traumatic experiences. Understanding these neurobiological mechanisms is essential for developing effective interventions and treatments aimed at mitigating the long-term impact of trauma on individuals' mental health and well-being.

Despite this, don’t despair! There are a range of proven strategies to help reduce and manage symptoms, many of which are in the pages to follow.

PTSD and the DSM-V-TR

In the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Text Revision (DSM-V-TR), the criteria for Post-Traumatic Stress Disorder (PTSD) include exposure to a traumatic event, persistent re-experiencing of the trauma through intrusive memories or flashbacks, avoidance of trauma-related stimuli, negative alterations in cognition and mood, and heightened arousal and reactivity. These symptoms must persist for more than one month and cause significant distress or impairment in functioning (American Psychiatric Association, 2013). In the United States, the prevalence of PTSD varies depending on factors such as population demographics and exposure to trauma. According to the National Institute of Mental Health (NIMH), approximately 7-8% of the US population will experience PTSD at some point in their lives ("Post-Traumatic Stress Disorder," 2021). In Australia, the prevalence of PTSD is estimated to be around 4.4%, with rates slightly higher among females compared to males (Australian Bureau of Statistics, 2008). These statistics highlight the significant impact of PTSD on individuals' mental health and the

importance of early intervention and support for those affected by trauma.

Complex Trauma/PTSD

Complex Post-Traumatic Stress Disorder (C-PTSD) is a psychological condition that arises from exposure to prolonged, severe, or repeated trauma, particularly in interpersonal relationships where there is an imbalance of power. While similar to Post-Traumatic Stress Disorder (PTSD), C-PTSD is distinguished by its broader array of symptoms and the chronic nature of the trauma experienced. Individuals with C-PTSD often grapple with profound difficulties in regulating emotions, forming and maintaining relationships, and establishing a stable sense of self. This condition can have far-reaching impacts on every aspect of an individual's life, affecting their mental, emotional, and physical well-being.

Unlike PTSD, which typically results from a single traumatic event or a series of discrete events, C-PTSD develops in response to prolonged or repeated trauma, often occurring within interpersonal relationships characterized by power dynamics, such as childhood abuse, neglect, or domestic violence. Additionally, individuals who have experienced prolonged exposure to adverse conditions such as captivity, human trafficking, or systemic oppression may also develop C-PTSD. The chronic and pervasive nature of the trauma experienced in C-PTSD can lead to profound disruptions in the individual's sense of safety, trust, and autonomy, resulting in a complex array of symptoms that extend beyond those traditionally associated with PTSD.

Diagnostic Symptoms

While C-PTSD is not formally recognized by the DSM-V-TR, it is codified by the International Classification of Diseases (Eleventh Revision) (ICD-11). According to to the ICD-11, the following criteria are applicable to C-PTSD:

Exposure to Prolonged Trauma: The individual has been exposed to prolonged, severe, or repeated trauma, typically beginning in childhood or early adolescence, and continuing over an extended period.

Symptoms of PTSD: In addition to the core symptoms of PTSD, such as intrusive memories, avoidance of trauma-related stimuli, negative alterations in mood and cognition, and heightened arousal and reactivity, individuals with C-PTSD may also experience disturbances in self-organization.

Disturbances in Self-Organization: These disturbances may manifest as difficulties in regulating emotions, maintaining a coherent sense of self, forming secure attachments, and navigating interpersonal relationships.

Significant Impairment in Functioning: The symptoms of C-PTSD must cause significant distress or impairment in social, occupational, or other important areas of functioning.

The diagnosis of Complex PTSD acknowledges the unique challenges faced by individuals who have experienced prolonged or repeated trauma, highlighting the need for tailored interventions and support to address the complex and multifaceted nature of their symptoms.

Other Conceptualisations

In his seminal work, ***CPTSD: From Surviving to Thriving***, Pete Walker names the following 17 experiences as foundational to C-PTSD:

Emotional dysregulation: Struggling with intense and fluctuating emotions, making it challenging to manage or stabilise feelings.
Emotional flashbacks: Intense emotional experiences that overwhelm the present, often leading to dissociation and feeling as though one is reliving past traumas.
Negative self-concept: Pervasive feelings of shame, guilt, or worthlessness, often stemming from prolonged abuse or invalidation.
Distorted perception of the perpetrator: Oscillating between idealising and demonising the perpetrator, struggling to form a clear understanding of the abuser's role.
Chronic feelings of emptiness: A persistent sense of inner void or emotional numbness, leading to difficulty in experiencing joy or fulfilment.

Interpersonal difficulties: Challenges in forming and maintaining relationships due to trust issues, fear of intimacy, or difficulty establishing healthy boundaries.

Hypervigilance: Heightened sensitivity to perceived threats, leading to constant alertness or readiness for danger, even in safe environments.

Sleep disturbances: Insomnia, nightmares, or disrupted sleep patterns stemming from recurring traumatic memories.

Difficulty with emotional attachment: Struggles in forming secure and healthy attachments, either being overly clingy or emotionally distant in relationships.

Somatic symptoms: Physical manifestations of distress, such as chronic pain, gastrointestinal issues, or tension, reflecting unresolved trauma stored in the body.

Sense of foreshortened future: Difficulty envisioning or planning for the future due to a pervasive sense of hopelessness or belief that life will be cut short.

Self-destructive behaviour: Engaging in harmful actions, from substance abuse to self-harm, as maladaptive coping mechanisms to manage emotional pain.

Difficulty regulating self-esteem: Fluctuations in self-worth, swinging between feeling excessively competent or inadequate, often in response to external validation or criticism.

Impaired sense of agency: Feeling powerless or lacking control over one's life, stemming from the history of trauma and an environment where choices were limited or disregarded.

Cognitive distortions: Persistent negative beliefs about oneself, others, and the world, leading to skewed perceptions and interpretations of events.

Survival strategies: Adaptive behaviours developed in response to trauma, which might have been crucial for survival but become hindrances in daily life.

Difficulty with emotional expression: Challenges in articulating emotions or needs, possibly due to a history of invalidated feelings or repression.

What does recovery look like?

We'll be exploring recovery in great detail in future modules, but for now, it would be fair to say that recovery is a challenging undertaking, but one that is absolutely worth it. Like most recovery journeys, yours is not likely to be linear; that is, it's not as easy as 'point A to point B'. There will be twists, turns, trials and many tribulations, but if you stick with it I have little doubt that you will be able to bring some much needed and lasting relief.

Put simply, recovery is represented by less frequent emotional flashbacks (more on these in the next module), and less intense symptoms when they do occur. You will feel more settled and calm in your relationships, and you will have a greater capacity for depth and vulnerability. Your body, more specifically your nervous system, will be less easily activated, and you'll find you can handle more than you were able to previously. You'll have a deeper, more positive relationship with your self, leading to better social, occupational and romantic opportunities.

I'm not sure about you, but the above sounds like a pretty good reason to get started!

Toxic Shame

While all symptoms of C-PTSD are troubling in their own right, some a more entrenched than others, and toxic shame is no exception.

Toxic shame is a deeply ingrained emotional response that can have devastating effects on individuals, particularly those experiencing Complex Post-Traumatic Stress Disorder (C-PTSD). Stemming from repeated experiences of trauma, neglect, or abuse, toxic shame manifests as an overwhelming sense of being fundamentally flawed, unworthy, and unlovable. Unlike healthy shame, which serves as a signal that we've violated our own or societal standards, toxic shame goes beyond this function, permeating every aspect of a person's life and sense of self.

In the context of C-PTSD, where individuals have endured prolonged and often severe trauma, toxic shame can become a persistent companion, shaping their perceptions, behaviors, and relationships in profound ways. Here are some of the key ways in which toxic shame can negatively impact someone with C-PTSD:

Self-Loathing: Toxic shame leads individuals to internalize a belief that they are inherently defective or worthless. This pervasive self-loathing can erode self-esteem and contribute to feelings of hopelessness and despair.
Self-Isolation: Fearing judgment and rejection from others, individuals with toxic shame may withdraw from social interactions, convinced that they are unworthy of connection or belonging. This isolation can exacerbate feelings of loneliness and deepen the sense of alienation.
Perfectionism: In an attempt to avoid the painful feelings associated with shame, individuals may develop perfectionistic tendencies, striving relentlessly for flawlessness in all areas of their lives. However, this pursuit is often fueled by an underlying fear of failure and a desperate desire for validation and acceptance.
Maladaptive Coping Mechanisms: To numb the intensity of their shame, individuals may turn to unhealthy coping mechanisms such as substance abuse, self-harm, or compulsive behaviors. These coping strategies provide temporary relief but ultimately perpetuate the cycle of shame and self-destructive patterns.
Impaired Relationships: Toxic shame can sabotage intimate relationships, as individuals may struggle to trust others and reveal their true selves for fear of rejection or abandonment. This fear of intimacy can lead to emotional distancing, conflicts, and ultimately, the breakdown of relationships.
Emotional Dysregulation: Shame often coexists with intense emotions such as anger, guilt, and anxiety, contributing to emotional dysregulation. Individuals may experience frequent mood swings, outbursts of rage, or episodes of dissociation as they struggle to cope with overwhelming feelings of shame and inadequacy.
Difficulty Seeking Help: Shame can act as a barrier to seeking help or support, as individuals may perceive reaching out for assistance as an admission of weakness or failure. This reluctance to seek help can prolong suffering and hinder the healing process.

Addressing toxic shame in the context of C-PTSD requires a comprehensive therapeutic approach that focuses on building self-compassion, challenging negative beliefs, and fostering a sense of safety and validation. Through trauma-informed therapy, individuals can begin to unravel the layers of shame, reclaim their sense of self-worth, and cultivate healthier ways of relating to themselves and others. Healing from toxic shame is a gradual and nonlinear journey, but with patience, support, and perseverance, individuals can learn to break free from its grip and reclaim their lives (See **Module 4**).

The Perils of Misdiagnosis

Misdiagnosis is a pervasive issue in the realm of mental health, often leading to profound consequences for individuals seeking understanding and support for their struggles. This is especially true for individuals grappling with Complex Post-Traumatic Stress Disorder (CPTSD), a condition stemming from prolonged exposure to traumatic events or circumstances, typically involving interpersonal trauma. The ramifications of a misdiagnosis on someone with CPTSD extend far beyond mere inconvenience; they can exacerbate symptoms, prolong suffering, and hinder the healing process. Let's delve into the multifaceted impact a misdiagnosis can have on those with

CPTSD.

Prolonged Suffering: Misdiagnosis can result in individuals with CPTSD enduring prolonged suffering due to the lack of appropriate treatment and understanding. Instead of receiving interventions tailored to their specific needs, they may undergo treatments targeting different conditions, which fail to address the underlying trauma at the core of their symptoms. Without proper recognition and treatment of CPTSD, individuals may continue to struggle with debilitating symptoms such as flashbacks, emotional dysregulation, and interpersonal difficulties, perpetuating their distress.

Invalidating experiences: Being misdiagnosed can lead individuals with CPTSD to feel invalidated and misunderstood. When their symptoms are attributed to a different mental health condition, they may internalize a sense of fault or inadequacy, believing that their struggles are a result of personal shortcomings rather than the profound impact of trauma. This invalidation can exacerbate feelings of shame, self-doubt, and isolation, further complicating their journey toward healing.

Treatment barriers: Misdiagnosis can erect significant barriers to accessing appropriate treatment and support. Individuals may find themselves caught in a cycle of ineffective treatments, unable to make meaningful progress toward recovery. Moreover, misdiagnosis can contribute to skepticism or mistrust of mental health professionals, making it challenging for individuals to seek help or engage in therapeutic interventions. Without accurate diagnosis and targeted treatment approaches, individuals with CPTSD may feel disheartened and disillusioned, impeding their willingness to pursue avenues of healing.

Heightened vulnerability: The failure to identify CPTSD can leave individuals vulnerable to further trauma and retraumatization. Without understanding the root cause of their symptoms, individuals may unknowingly engage in behaviors or relationships that perpetuate patterns of abuse and exploitation, exacerbating their condition and deepening their emotional wounds. This heightened vulnerability underscores the critical importance of accurate diagnosis in guiding individuals toward environments and relationships that foster healing and safety.

Module 1.2: Origins of C-PTSD

THE GENESIS OF CPTSD

Complex Post-Traumatic Stress Disorder (CPTSD) is a profound and multifaceted condition that arises from a complex interplay of biological, psychological, and environmental factors. To truly understand its origins, we must embark on a journey that traverses the realms of attachment theory, evolutionary psychology, and the diverse array of traumatic experiences that culminate in its manifestation.

Attachment Theory: The Building Blocks of Human Connection

At the heart of CPTSD lies the intricate web of attachment bonds formed in infancy and early childhood. Attachment theory, pioneered by John Bowlby and further developed by Mary Ainsworth, elucidates how the quality of these . early relationships shapes an individual's capacity for emotional regulation, self-esteem, and interpersonal functioning throughout life. Secure attachment, characterized by consistent caregiving and responsiveness to a child's needs, lays the foundation for healthy emotional development. Conversely, insecure attachment, marked by neglect, abuse, or inconsistent caregiving, can set the stage for the emergence of CPTSD symptoms later in life

At its core, attachment theory posits that human beings are biologically predisposed to seek proximity to caregivers in times of distress as a means of promoting survival and emotional regulation. These early attachment bonds not only influence individuals' internal working models of themselves and others but also lay the groundwork for their ability to form and

maintain relationships throughout life. When examining the emergence of Complex Post-Traumatic Stress Disorder (CPTSD), attachment theory offers valuable insights into the profound impact of early relational experiences on individuals' vulnerability to trauma and subsequent symptomatology.

Secure vs. Insecure Attachment: The Foundation of Emotional Security.

When examining the emergence of Complex Post-Traumatic StressAttachment theory delineates between secure and insecure attachment styles, each characterized by distinct patterns of behavior and affect regulation. In the context of secure attachment, caregivers provide consistent, responsive, and nurturing care, fostering a sense of safety, trust, and emotional security in the child. These secure base experiences serve as a foundation for the development of adaptive coping mechanisms, self-esteem, and resilience in the face of adversity. In contrast, insecure attachment arises from disruptions in caregiving, manifesting as anxious-preoccupied, dismissive-avoidant, or fearful-avoidant attachment styles. These insecure attachment patterns are associated with heightened vulnerability to stress, emotional dysregulation, and difficulties in interpersonal relationships, predisposing individuals to the development of CPTSD later in life. Disorder (CPTSD), attachment theory offers valuable insights into the profound impact of early relational experiences on individuals' vulnerability to trauma and subsequent symptomatology.

Attachment Trauma: The Erosion of Emotional Safety

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Impact on Symptoms: The Relationship Between Attachment and Trauma

The relationship between attachment and trauma is intricately intertwined, with early attachment experiences shaping individuals' responses to subsequent traumatic events and their ability to cope with distress. For individuals with CPTSD, insecure attachment patterns may predispose them to engage in maladaptive coping strategies such as dissociation, emotional numbing, or self-destructive behaviors as a means of regulating overwhelming emotions and maintaining a semblance of control. Moreover, the absence of secure attachment figures exacerbates feelings of social isolation, exacerbating the sense of alienation and disconnection characteristic of CPTSD symptomatology.

Healing Through Secure Attachments

In light of the profound impact of attachment on the emergence of CPTSD, therapeutic interventions must prioritize the establishment of secure, attuned relationships between individuals and their caregivers or mental

health professionals. Trauma-informed approaches such as Attachment-Based Therapy, EMDR (Eye Movement Desensitization and Reprocessing), and Sensorimotor Psychotherapy aim to repair disrupted attachment bonds, cultivate emotional regulation skills, and foster a sense of safety and trust in the therapeutic relationship. By addressing the underlying attachment wounds that contribute to CPTSD symptomatology, individuals can embark on a journey of healing, resilience, and relational empowerment.

Navigating the Complex Terrain of Attachment and Trauma

In summary, attachment theory provides a powerful lens through which to understand the emergence of CPTSD and the profound impact of early relational experiences on individuals' vulnerability to trauma. By elucidating the interplay between attachment patterns, trauma exposure, and symptomatology, we gain insight into the complex etiology of CPTSD and the critical role of secure relationships in promoting healing and resilience. Armed with this understanding, mental health professionals can tailor interventions that honor the centrality of attachment in individuals' lived experiences, fostering a sense of safety, trust, and belonging essential for navigating the complexities of trauma recovery.

Insights from Evolutionary Psychology

Complex Post-Traumatic Stress Disorder (CPTSD) is a condition that reflects the profound impact of prolonged or repetitive trauma on an individual's psychological and physiological well-being. To comprehend the origins of CPTSD, one must delve into the evolutionary underpinnings of human behavior and cognition, as elucidated by evolutionary psychology. This interdisciplinary field offers valuable insights into the adaptive significance of trauma responses encoded in human biology, shedding light on the multifaceted nature of CPTSD.

Adaptive Reponses to Stress: An Evolutionary Perspective

Evolutionary psychology posits that human beings, like other organisms, have evolved psychological mechanisms shaped by natural selection to navigate the challenges of survival and reproduction. Central to this perspective is the recognition that threats to survival have been pervasive throughout human history, ranging from predatory encounters to interpersonal violence within social groups. In response to such threats, individuals have developed a repertoire of adaptive responses aimed at maximizing their chances of survival and reproductive success.

Hypervigilance and Threat Detection

One principle of evolutionary psychology implicated in the emergence of CPTSD is the heightened sensitivity to potential threats, known as hypervigilance. From an evolutionary standpoint, hypervigilance evolved as a protective mechanism to detect and respond to potential dangers in the environment. However, in the context of chronic or interpersonal trauma, this adaptive response can become dysregulated, leading to persistent feelings of anxiety, hyperarousal, and a pervasive sense of danger.

Stress Response and Allostatic Load

Evolutionary psychology also highlights the role of stress response systems in mediating individuals' responses to trauma and adversity. When confronted with a perceived threat, the body initiates a cascade of physiological responses, including the release of stress hormones such as cortisol and activation of the sympathetic nervous system. While these responses are adaptive in the short term, chronic or severe trauma can lead to dysregulation of stress response systems, resulting in allostatic load—the cumulative wear and tear on the body's physiological

systems. Allostatic load is implicated in a range of health problems, including cardiovascular disease, metabolic disorders, and psychiatric conditions such as CPTSD.

Bridging Evolutionary Insights and Trauma Recovery

In conclusion, evolutionary psychology provides a valuable framework for understanding the origins of CPTSD and the adaptive significance of trauma responses encoded in human biology. By elucidating the interplay between hypervigilance, attachment, stress response systems, and allostatic load, we gain insight into the complex etiology of CPTSD and the enduring legacy of evolutionary pressures on human behavior and cognition. Armed with this understanding, mental health professionals can tailor interventions that honor the adaptive functions of trauma responses while promoting healing, resilience, and empowerment in individuals affected by CPTSD. Ultimately, by bridging the gap between evolutionary insights and trauma recovery, we can strive towards a more comprehensive understanding of human suffering and the pathways to healing in the aftermath of trauma.

Trauma in all its Forms

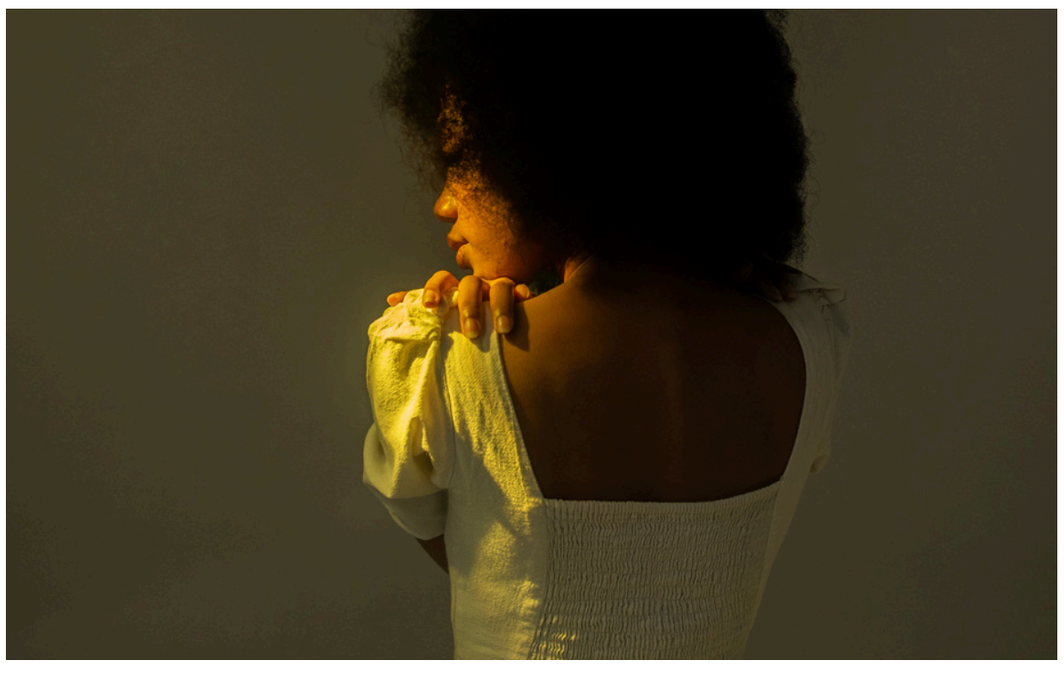
Childhood Abuse and Neglect: Physical, emotional, or sexual abuse inflicted by caregivers or authority figures during critical periods of development can undermine a child's sense of safety, trust, and self-worth, laying the groundwork for CPTSD.

Attachment Trauma: Disruptions in early attachment bonds due to parental abandonment, neglect, or unresolved loss can engender profound feelings of insecurity and instability, perpetuating patterns of relational dysfunction and emotional distress.

Interpersonal Violence: Persistent exposure to interpersonal violence, whether in the form of domestic abuse, intimate partner violence, or community violence, can erode an individual's sense of safety and agency, fostering a chronic state of hyperarousal and vigilance characteristic of CPTSD.

Institutional Trauma: Traumatic experiences within institutional settings such as foster care, incarceration, or war zones can exacerbate feelings of powerlessness, betrayal, and social isolation, amplifying the risk of CPTSD among affected individuals.

Sexual Violence: Trauma arising from sexual violence encompasses a spectrum of psychological and emotional distress resulting from non-consensual sexual acts. Survivors may experience profound feelings of shame, guilt, and fear, along with symptoms of post-traumatic stress disorder (PTSD) such as flashbacks, hypervigilance, and avoidance behaviors. Long-term consequences can include difficulties in interpersonal relationships, sexual dysfunction, and psychological disorders. Effective support and trauma-informed interventions are essential for survivors to heal and regain a sense of safety and empowerment.



END OF MODULE 1

Module 2.1: Trauma Response Types

THE 4 TRAUMA RESPONSE TYPES

The 4 F's

The concept of the **"4 F's"** in trauma response, also known as the **"4 Fs of Survival,"** refers to four common types of coping strategies that individuals may adopt when faced with traumatic experiences. These responses are often understood within the framework of evolutionary psychology and are deeply ingrained in human behavior. Each response is associated with specific patterns of behavior, physiological reactions, and psychological manifestations. While they are not formally recognized in the DSM-V-TR (Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Text Revision), they provide valuable insights into how individuals respond to overwhelming stress and adversity.

The Fight Type

Description: The fight response is characterized by a proactive and assertive approach to dealing with threats. Individuals exhibiting this response may confront challenges head-on, refusing to back down or submit. They often display aggression, anger, and defiance as a means of asserting control over their environment.

Behaviors: Aggression, confrontation, resistance, assertiveness, impulsivity, irritability.

Reflective of: Conditions such as Intermittent Explosive Disorder, Conduct Disorder, Oppositional Defiant Disorder.

The Freeze Type

Description: The freeze response entails a state of immobilization or shutdown in the face of overwhelming stress. Individuals experiencing this response may feel paralyzed, unable to act or make decisions. They may dissociate from their surroundings as a means of coping with the perceived threat.
Behaviors: Inhibition, numbness, dissociation, apathy, passivity, feeling disconnected from reality.
Reflective of: Conditions such as Acute Stress Disorder, Post-Traumatic Stress Disorder (PTSD), Dissociative Disorders.

The Flight Type

Description: The flight response involves escaping or avoiding perceived threats. Individuals exhibiting this response may seek to physically remove themselves from the source of danger or emotionally detach from distressing situations. They prioritize evasion and avoidance as strategies for self-preservation.
Behaviors: Avoidance, escapism, withdrawal, dissociation, restlessness, seeking distractions.
Reflective of: Conditions such as Panic Disorder, Generalized Anxiety Disorder, Avoidant Personality Disorder.

The Fawn Type

Description: The fawn response involves seeking to appease or please others as a way of avoiding harm. Individuals exhibiting this response may prioritize the needs and desires of others over their own, often at their own expense. They may engage in submissive behavior and seek validation and approval from others.
Behaviors: People-pleasing, submissiveness, compliance, seeking reassurance, difficulty asserting boundaries.
Reflective of: Conditions such as Dependent Personality Disorder, Borderline Personality Disorder, Avoidant Personality Disorder.

It's important to note that individuals may not exclusively exhibit one trauma response type, and responses can vary depending on the context and severity of the trauma. Additionally, these responses are adaptive mechanisms developed over time to survive threatening situations, but they can also become maladaptive if they persist long after the trauma has ended or if they interfere with daily functioning. Understanding these trauma responses can inform therapeutic approaches aimed at promoting resilience and recovery in individuals who have experienced trauma.

Factors Influencing which Type we Develop

The development of the four trauma response types, or the 4 F's, is influenced by a combination of factors including individual characteristics of the victim and the nature of the trauma experienced. While these responses are shaped by evolutionary and biological processes, they are also influenced by psychological, social, and environmental factors. Let's delve into how each trauma type may develop based on these variables:

Fight Response:

Individual Characteristics: Individuals who have a strong sense of self-efficacy, assertiveness, or a history of facing challenges with aggression may be more inclined towards a fight response. Personality traits such as high levels of extroversion or impulsivity may also contribute to this response.
Type of Trauma: Situations where the individual perceives a direct threat to their physical or emotional integrity can trigger a fight response. For example, someone who has experienced physical abuse or bullying may develop a tendency to respond with aggression as a means of self-protection.

Goals of Recovery

Channeling Aggression: One of the primary goals for individuals with a fight response is to learn healthy ways of expressing and managing aggression. This involves teaching assertiveness skills, conflict resolution techniques, and anger management strategies. Addressing Underlying Trauma: It's essential to

explore the underlying causes of the fight response, which may stem from past experiences of violence, abuse, or trauma. Therapy can help individuals process these experiences and develop coping mechanisms that don't rely solely on aggression. Building Emotional Regulation: Learning to identify and regulate intense emotions is key to recovery. Techniques such as mindfulness, relaxation exercises, and cognitive-behavioral therapy (CBT) can be beneficial in developing emotional resilience and self-control.

The Flight Response

Individual Characteristics: People who are predisposed to anxiety, fearfulness, or have a history of trauma-related avoidance behaviors may be more prone to a flight response.
Traits such as high neuroticism or a tendency towards escapism can also contribute.
Type of Trauma: Traumatic events that evoke a sense of overwhelming danger or helplessness may lead to a flight response. For instance, individuals who have experienced natural disasters, serious accidents, or sudden loss may develop a pattern of avoiding situations that remind them of the trauma.

Recovery Goals

Gradual Exposure: For individuals with a flight response, the goal is to gradually confront and overcome avoidance behaviors. Therapeutic techniques such as exposure therapy can help desensitize individuals to triggers associated with the trauma, allowing them to reclaim a sense of safety and control.
Coping Skills Development: Building effective coping skills is essential for managing anxiety and stress associated with the flight response. This may involve teaching relaxation techniques, problem-solving skills, and strategies for challenging negative thought patterns.
Addressing Safety Concerns: Creating a safe and supportive environment is paramount in recovery. This may involve establishing boundaries, developing a support network, and ensuring access to resources for managing crisis situations.

Freeze Response:

Individual Characteristics: Individuals who have a history of trauma or have experienced chronic stress may be more likely to exhibit a freeze response. People with a tendency towards dissociation, emotional numbing, or difficulties with emotional regulation may also be predisposed.
Type of Trauma: Trauma that is sudden, severe, or perceived as life-threatening can trigger a freeze response. Examples include experiences such as sexual assault, combat trauma, or witnessing a traumatic event where the individual feels powerless to escape or defend themselves.

Recovery Goals:

Grounding Techniques: Individuals with a freeze response often benefit from grounding techniques that help reconnect them with the present moment and their surroundings. These may include sensory-based exercises, mindfulness practices, and techniques that promote body awareness.
Processing Traumatic Memories: Therapy focused on processing traumatic memories can help individuals move beyond the state of immobilization and begin to integrate the traumatic experience into their life story. This may involve approaches such as Eye Movement Desensitization and Reprocessing (EMDR) or trauma-focused cognitive-behavioral therapy (TF-CBT).
Reclaiming Agency: Empowering individuals to reclaim a sense of agency and control over their lives is central to recovery from the freeze response. This may involve setting achievable goals, building self-esteem, and fostering a sense of autonomy in decision-making.

Fawn Response:

Individual Characteristics: People who have a strong need for social approval, have experienced interpersonal trauma, or grew up in environments where their needs were consistently invalidated may develop a fawn response. Traits such as high agreeableness or a history of codependent relationships may also contribute.
Type of Trauma: Trauma that involves interpersonal harm or betrayal, such as emotional abuse, neglect, or exploitation, can lead to a fawn response. Individuals may learn

to prioritize the needs of others as a way of seeking safety or avoiding conflict.

Recovery Goals:

Boundary Setting: Establishing healthy boundaries is essential for individuals with a fawn response to protect their own needs and well-being. Therapy can help individuals recognize and assert their boundaries in relationships and other areas of life.

Self-Compassion: Cultivating self-compassion and self-care practices is vital for individuals who have learned to prioritize the needs of others over their own. This may involve developing a nurturing inner dialogue, practicing self-care activities, and setting aside time for relaxation and rejuvenation.

Assertiveness Training: Learning assertiveness skills is key to overcoming patterns of people-pleasing and passive behavior associated with the fawn response. Therapy can help individuals assert their needs and preferences in a respectful and assertive manner, enhancing their sense of self-worth and empowerment.

The Importance of Knowing your 4F Type

Self-Awareness:

Recognizing Patterns: Identifying your primary trauma response type can help you recognize patterns of behavior, thoughts, and emotions that may be linked to past traumatic experiences. This awareness allows you to gain insight into why you react to certain situations in specific ways.

Understanding Triggers: Knowing your trauma type can help you identify triggers that activate your trauma response. By understanding what triggers your fight, flight, freeze, or fawn response, you can take proactive steps to manage and mitigate these triggers in your daily life.

Tailored Treatment:

Personalized Interventions: Different trauma responses may require different therapeutic approaches. By knowing your trauma type, you and your therapist can tailor treatment interventions to address your specific needs and challenges effectively.

Targeted Coping Strategies: Understanding your trauma type allows you to implement targeted coping strategies that align with your response style. For example, if you tend to freeze in response to trauma, learning grounding techniques and practicing mindfulness may be particularly beneficial.

Empowerment:

Taking Control: Knowing your trauma type empowers you to take an active role in your healing journey. Rather than feeling overwhelmed or controlled by your trauma responses, you can develop agency and agency and take proactive steps towards recovery.

Building Resilience: Understanding how you respond to trauma can help you develop resilience by leveraging your strengths and addressing areas of vulnerability. With this knowledge, you can build adaptive coping skills that promote long-term well-being.

Improved Relationships:

Enhanced Communication: Communicating your trauma response type to trusted loved ones can foster understanding and empathy in your relationships. It allows others to support you more effectively and helps prevent misunderstandings or conflicts that may arise from misinterpreting your behavior.

Setting Boundaries: Knowing your trauma type can help you establish and maintain healthy boundaries in relationships. By understanding your needs and triggers, you can communicate boundaries clearly and assertively, fostering healthier and more fulfilling connections.

Prevention and Preparedness:

Anticipating Challenges: Understanding your trauma type enables you to anticipate challenges and potential triggers in various situations. This awareness allows you to plan ahead and implement coping strategies to manage stressors effectively.

Building Resilience: Armed with knowledge about your trauma response type, you can actively engage in activities and practices that promote resilience and well-being. Whether it's seeking therapy, practicing self-care, or engaging in supportive communities, you can take proactive steps

to protect your mental health and mitigate the impact of future traumas.

In conclusion, knowing your 4F trauma type is an essential aspect of trauma recovery and self-care. It provides valuable insights into your behavioral patterns, triggers, and coping mechanisms, empowering you to take control of your healing journey, build resilience, foster healthier relationships, and prepare for future challenges. By embracing self-awareness and seeking support when needed, you can embark on a path towards healing, growth, and greater well-being

Module 2.2: Emotional Flashbacks

EMOTIONAL FLASHBACKS

What are they?

Emotional flashbacks are a hallmark feature of Complex Post-Traumatic Stress Disorder (C-PTSD), a condition that arises from prolonged exposure to traumatic events, particularly in interpersonal relationships or environments where escape is difficult. Unlike typical PTSD flashbacks, which involve vivid re-experiencing of past traumatic events, emotional flashbacks primarily involve the reactivation of intense emotional states associated with past trauma, without necessarily recalling the specific details of the traumatic event itself.

In the context of C-PTSD, emotional flashbacks can be triggered by a wide range of stimuli, including sensory cues, interpersonal interactions, or internal thoughts and emotions. These triggers often evoke feelings of fear, shame, helplessness, or profound sadness, which may be disproportionate to the current situation. Emotional flashbacks can be highly distressing and disruptive, impairing one's ability to function effectively in daily life and undermining their sense of safety and well-being. Understanding emotional flashbacks is essential in the context of C-PTSD for several reasons:

Recognition and Validation: Many individuals with C-PTSD may not initially recognize emotional flashbacks for what they are, mistaking them for irrational mood swings or inherent flaws in their personality. Learning about emotional flashbacks can help individuals validate their experiences and understand that their intense emotional reactions are rooted in past trauma rather than inherent deficiencies.

Enhanced Self-Awareness: Recognizing emotional flashbacks requires a heightened level of self-awareness and introspection. By learning to identify the signs and symptoms of emotional flashbacks, individuals can develop greater insight into their emotional triggers and patterns of reactivity, paving the way for more effective coping strategies and emotional regulation techniques.

Prevention of Escalation: Emotional flashbacks have the potential to escalate into more severe symptoms of C-PTSD, such as dissociation, self-harm, or substance abuse, if left unaddressed. By intervening early and implementing appropriate coping mechanisms, individuals can mitigate the impact of emotional flashbacks and prevent further escalation of symptoms.

Treatment Planning: Understanding the role of emotional flashbacks in C-PTSD is essential for developing targeted treatment interventions. Therapeutic approaches such as trauma-focused therapy, cognitive-behavioral therapy (CBT), dialectical behavior therapy (DBT), and mindfulness-based interventions can help individuals learn to recognize and manage emotional flashbacks more effectively, thereby facilitating the healing process and promoting greater resilience.

In summary, emotional flashbacks are a critical aspect of C-PTSD that warrant careful attention and understanding. By recognizing the signs and symptoms of emotional flashbacks, individuals can gain greater insight into their trauma-related experiences, develop effective coping strategies, and work towards healing and recovery. Educating oneself about emotional flashbacks is an important step in reclaiming agency and control over one's emotional well-being in the face of C-PTSD.

Types of Triggers

External Triggers:

Sensory Cues: Certain smells, sounds, sights, or tactile sensations reminiscent of the traumatic event can trigger emotional flashbacks. For example, the smell of a particular perfume may evoke memories of an abusive caregiver.

Environmental Stimuli: Being in environments similar to those where the trauma occurred, such as crowded spaces, dark rooms, or specific locations, can trigger emotional flashbacks.

Interpersonal Interactions: Interactions with individuals who resemble perpetrators or exhibit similar behaviors can trigger emotional flashbacks. For instance, encountering someone with a similar voice or facial features as the abuser may elicit a strong emotional reaction.

Internal Triggers:

Thoughts and Memories: Intrusive thoughts or memories related to the traumatic event can trigger emotional flashbacks. These may surface spontaneously or be triggered by stressors or reminders of the trauma.

Emotional States: Certain emotional states, such as feeling vulnerable, powerless, or overwhelmed, can trigger emotional flashbacks by activating patterns of emotional dysregulation associated with past traumas.

Potential Symptoms of Emotional Flashbacks

Emotional flashbacks can manifest in a wide range of symptoms, which may vary in intensity and duration. Some common symptoms include:

Intense Emotional Distress:

Overwhelming feelings of fear, shame, guilt, sadness, or anger that are disproportionate to the current situation. Difficulty identifying or understanding the source of the emotional distress, leading to confusion or disorientation.

Physical Sensations:

Somatic symptoms such as rapid heartbeat, sweating, trembling, dizziness, or nausea.
Hyperventilation or difficulty breathing, chest tightness, or a sensation of choking.

Cognitive Distortions:

Distorted perceptions of reality, including feelings of unreality or dissociation.
Intrusive thoughts, rumination, or flashbacks related to past traumatic events.

Behavioral Responses:
Avoidance behaviors aimed at escaping or minimizing triggers of emotional flashbacks.
Impulsive or self-destructive behaviors as a means of coping with overwhelming emotions.

Interpersonal Difficulties:
Difficulty forming or maintaining relationships due to mistrust, fear of abandonment, or difficulty establishing boundaries.
Heightened sensitivity to perceived criticism, rejection, or perceived threats in interpersonal interactions.

Regulatory Difficulties:
Challenges with emotion regulation, including difficulty tolerating distressing emotions or engaging in effective coping strategies.
Patterns of self-soothing or self-sabotaging behaviors to cope with emotional distress.

Impact on Functioning:
Impairment in daily functioning, including difficulties at work, school, or in social situations. Disruption of sleep patterns, including insomnia or nightmares related to past traumas.

It's important to note that individuals experiencing emotional flashbacks may not necessarily recognize them as such, as they may be confused or overwhelmed by the intensity of their emotions. However, increasing awareness and understanding of emotional flashbacks can empower individuals to seek appropriate support and develop effective coping strategies to manage their symptoms.

In conclusion, emotional flashbacks are a significant aspect of C-PTSD, impacting individuals' emotional well-being, relationships, and overall quality of life. By recognizing potential triggers and symptoms of emotional flashbacks, individuals can begin to navigate their experiences with greater insight and resilience, paving the way towards healing and recovery.

The ABC's of Recovery

It's important to note that finding the most effective strategies for relieving emotional flashbacks often involves a process of trial and error, and what works best may vary from person to person. Working with a qualified mental health professional can help individuals tailor their approach to meet their specific needs and circumstances, facilitating healing and recovery from C-PTSD - but seeing how this is a self-guided recovery ebook, a self-guided approach will be explored below:

Grounding

I know we're all weary from hearing about deep breathing and grounding strategies the umpteenth time, however they still hold their place. Grounding techniques can help individuals anchor themselves in the present moment and regain a sense of safety when experiencing emotional flashbacks.

Examples include:

- **5-4-3-2-1 Technique:** Name five things you can see, four things you can touch, three things you can hear, two things you can smell, and one thing you can taste.
- **Deep Breathing:** Focus on slow, deep breaths, counting each inhale and exhale to help regulate the nervous system.
- **Progressive Muscle Relaxation:** Tense and relax each muscle group in the body sequentially to release tension and promote relaxation.

Self-Soothing Activities:

Engaging in soothing activities can help calm the nervous system and reduce the intensity of emotional flashbacks. This may include:

- **Mindfulness Meditation:** Practice mindfulness techniques to cultivate present-moment awareness and acceptance of one's experiences.

- **Creative Expression:** Express emotions through art, music, journaling, or other creative outlets to process and release pent-up feelings.

- **Self-Compassion:** Practice self-compassion by speaking kindly to oneself, acknowledging difficult emotions without judgment, and offering oneself comfort and

care.

Safety Planning:

- Developing a safety plan can provide a sense of reassurance and preparedness during emotional flashbacks. This may involve:
  - **Creating a Safe Space:** Identify a physical or mental space where you feel safe and secure, and visualize yourself retreating to this space during flashbacks.
  - **Establishing Boundaries:** Set clear boundaries with others to protect your emotional well-being and minimize triggers for flashbacks.
  - **Emergency Contacts:** Compile a list of trusted friends, family members, or mental health professionals who you can reach out to for support during distressing episodes.
  -

Cognitive Strategies:

- Cognitive strategies can help challenge distorted thoughts and beliefs that contribute to emotional flashbacks. This may include:
  - **Thought Records:** Keep track of triggering thoughts and examine evidence for and against them to gain perspective and challenge negative beliefs.
  - **Cognitive Restructuring:** Replace distorted thoughts with more balanced and realistic alternatives, reframing the meaning of past traumatic experiences.
  - **Positive Affirmations:** Repeat affirmations or positive statements to counteract negative self-talk and cultivate self-esteem and resilience.

Lifestyle Modifications:

- Making healthy lifestyle choices can support overall well-being and resilience in managing emotional flashbacks. This may involve:
- Regular Exercise: Engage in physical activity to reduce stress, improve mood, and promote relaxation.
- Healthy Sleep Habits: Prioritize adequate sleep and establish a consistent sleep routine to enhance emotional regulation and coping abilities.
- Nutrition: Maintain a balanced diet rich in fruits, vegetables, whole grains, and lean proteins to support physical and mental health.

Social Support:

- Cultivating supportive relationships can provide comfort, validation, and encouragement during times of distress. This may involve:
  - **Connecting with Others:** Reach out to trusted friends, family members, support groups, or online communities for understanding and validation.
  - **Peer Support:** Seek out individuals who have experienced similar challenges with C-PTSD and emotional flashbacks to share experiences and coping strategies.
  - **Therapeutic Relationships:** Foster a trusting and collaborative relationship with a qualified mental health professional who can provide guidance, validation, and evidence-based interventions tailored to your needs.

It's important for individuals to experiment with these strategies and determine which ones resonate most effectively with their unique preferences and experiences.

What to do in a Flashback

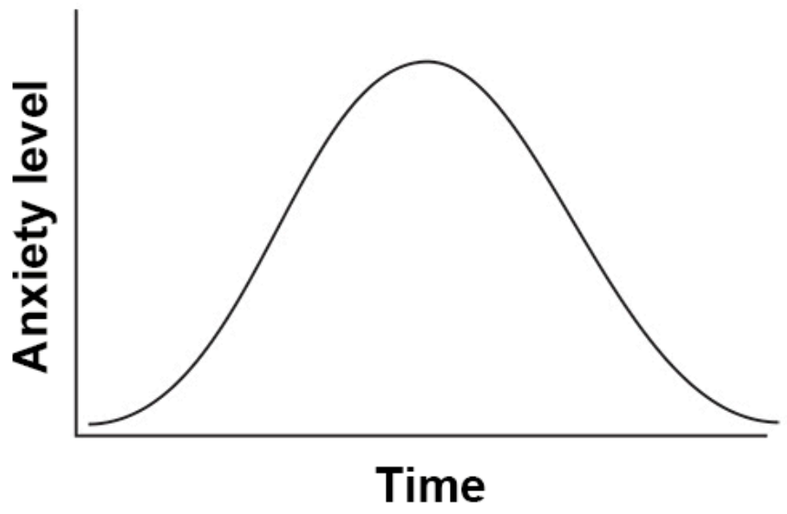
- The first and most important response to a flashback is to actually label the experience for what is is: **“I am having an emotional flashback”**. These temporary episodes have the capacity transport you to a timeless aspect of the mind, evoking the same sense of helplessness, hopelessness, and peril you felt in childhood. Understand that the emotions and sensations you are feeling are ***echoes of past memories, and they hold no power to harm you in the present moment.***

- Tell yourself: **"Though I feel fear, I am not at risk! I am secure in the present moment."** Acknowledge that you are currently in a safe environment, ***free from the perils of the past.***

- Imagine a younger, more fragile version of yourself. **Offer comforting words to this Inner Child.** Let the child understand that you love them unconditionally – ***that they can always turn to you for solace and safety when they feel bewildered and frightened.***

- Remember that you **inhabit an adult body now, equipped with allies, abilities, and resources to safeguard you**—resources that were unavailable to you as a child. ***Feeling vulnerable and small signals that you may be experiencing a flashback.***

- Experience the sensation of fear within your body **without immediately responding to it**. Fear is merely an energy present in your body. ***It holds no power to harm you as long as you refrain from fleeing from it.***



The above is the lifecycle of anxiety.

If we try to compulsively control the anxiety, we're in effect signaling to our brain that this experience is dangerous and threatening, and that we must remain on constant alert to protect ourselves. Instead, if we wait until the anxiety has subsided back to 'baseline', which is our anxiety level prior to a flashback, we are showing the brain that all we're experiencing is a temporary and discomforting emotional experience, and that ***all we really need to do is handle it with patience and grace.***

- **Combat the Inner Critic's tendency to magnify and catastrophize situations** (more on the inner critic in **Module 3**). Employ Thought-stopping to interrupt the critic's relentless exaggeration of threats and its futile attempts to control the uncontrollable. ***Reject any inclination to shame, despise, or forsake yourself.*** Direct the anger stemming from self-attack towards firmly rejecting the critic's unjust self-condemnation. Utilize Thought-substitution and Thought-correction techniques to ***swap negative thoughts with a predetermined list of your strengths and achievements.***

- **Nurture secure connections and reach out for assistance when needed.** Allow yourself solitude when necessary, but don't allow shame to drive you into isolation. Experiencing shame does not equate to being shameful. ***Educate your close relationships about emotional flashbacks and request their support in navigating and processing them together.***

- Develop the ability to recognize the various triggers that precipitate emotional flashbacks. Steer clear of individuals, environments, activities, and thoughts that evoke feelings of unsafety **until you're ready to face them gradually.** ***Implement proactive measures to manage triggering situations if avoidance is not feasible.***

END OF MODULE 2

Module 3.1: The Inner Critic

JUDGE, JURY & EXECUTIONER

What is the Inner Critic?

In the complex landscape of trauma recovery, the concept of the inner critic emerges as a significant psychological phenomenon, particularly within the realm of Complex Post-Traumatic Stress Disorder (C-PTSD). C-PTSD, resulting from prolonged exposure to trauma, often in interpersonal relationships or environments where escape is challenging, encompasses a myriad of symptoms and challenges that extend far beyond the scope of typical PTSD. Central to the experience of individuals with C-PTSD is the presence of an internalized voice known as the inner critic.

The inner critic represents an internalized version of critical or abusive voices from the past, typically stemming from childhood experiences of neglect, abuse, or invalidation. It manifests as an inner dialogue characterized by harsh self-judgment, self-blame, and self-doubt. Unlike constructive self-criticism, which can be adaptive and growth-oriented, the inner critic in C-PTSD tends to be relentless, punitive, and destructive, exacerbating feelings of worthlessness, shame, and inadequacy.

Understanding the inner critic is crucial in the context of C-PTSD for several reasons. Firstly, it serves as a pervasive barrier to healing and recovery, perpetuating cycles of self-sabotage, self-harm, and maladaptive coping strategies. Individuals with C-PTSD may internalize the negative messages of their inner critic, reinforcing core beliefs of unworthiness and powerlessness.

Moreover, the inner critic often intersects with other symptoms of C-PTSD, such as emotional

flashbacks, dissociation, and difficulties with emotional regulation. During emotional flashbacks, the inner critic may amplify feelings of fear, shame, and helplessness, exacerbating the intensity and duration of these distressing experiences. Similarly, in moments of dissociation, the inner critic may fuel feelings of detachment and disconnection, further alienating individuals from their sense of self and reality.

Recognizing and addressing the inner critic is thus essential in C-PTSD recovery. By cultivating self-awareness and understanding the origins and mechanisms of the inner critic, individuals can begin to challenge and reframe negative self-talk, replacing it with self-compassion, self-validation, and self-empowerment. Therapeutic interventions, such as cognitive-behavioral therapy (CBT), dialectical behavior therapy (DBT), and inner child work, can provide individuals with tools and strategies to navigate and disarm the inner critic effectively.

In essence, the concept of the inner critic in the context of C-PTSD underscores the profound impact of past trauma on one's sense of self and inner world. By addressing and transforming the critical voices within, individuals with C-PTSD can embark on a journey of self-discovery, healing, and reclaiming their innate worth and resilience.

Origins of the Inner Critic?

In the intricate tapestry of Complex Post-Traumatic Stress Disorder (C-PTSD), the development and persistence of the inner critic represent a significant psychological phenomenon deeply intertwined with attachment theory. C-PTSD, stemming from prolonged exposure to trauma, particularly in interpersonal relationships or environments where escape is challenging, sets the stage for the emergence of critical self-dialogue that can profoundly impact one's sense of self-worth, safety, and identity.

The inner critic in C-PTSD can be seen as an internalized version of critical or abusive voices from the past, primarily stemming from childhood experiences. Children growing up in environments marked by trauma may internalize messages of unworthiness, shame, and inadequacy, whether through direct verbal abuse, emotional neglect, or witnessing dysfunctional patterns of interaction between caregivers. These negative messages become embedded within the individual's psyche, shaping their self-concept and worldview well into adulthood.

The inner critic often emerges as a maladaptive coping mechanism in response to the perceived threat of abandonment, rejection, or harm. In an attempt to anticipate and mitigate potential dangers, individuals with C-PTSD may adopt an internalized voice that mirrors the critical or abusive messages received from caregivers. This inner critic serves as a form of self-protection, albeit a misguided one, by preemptively denigrating the self in an effort to avoid external criticism or punishment.

Furthermore, the inner critic is intricately linked to attachment-related schemas and working models of self and others. Individuals with insecure attachment styles, such as anxious-preoccupied or fearful-avoidant, are particularly vulnerable to the development of a harsh inner critic. For example, those with an anxious-preoccupied attachment style, characterized by hypervigilance to signs of rejection or abandonment, may internalize beliefs of being unlovable or inadequate, fueling the inner critic's relentless scrutiny and self-blame.

Similarly, individuals with a fearful-avoidant attachment style, torn between a desire for intimacy and a fear of vulnerability, may internalize conflicting messages about their worthiness and lovability, leading to heightened self-criticism and distrust of others. In both cases, the inner critic serves as a defensive mechanism aimed at regulating overwhelming emotions and maintaining a semblance of control in the face of relational uncertainty and perceived threats.

Understanding the development of the inner critic within the framework of attachment theory underscores the profound impact of early relational experiences on the formation of self-concept and interpersonal functioning.

Healing from C-PTSD involves not only addressing the overt symptoms of trauma but also unraveling the layers of internalized shame, self-blame, and self-doubt perpetuated by the inner critic. By fostering a compassionate and secure internal dialogue, individuals can begin to challenge and transcend the limiting beliefs instilled by past trauma, forging a path towards self-acceptance, resilience, and authentic connection with others.

How to breakdown the inner critic

Navigating the relentless inner critic can be a daunting aspect of healing from Complex Post-Traumatic Stress Disorder (C-PTSD). However, there are several effective self-help techniques individuals can utilize to quieten the critical voice and cultivate self-compassion and resilience. While these techniques are not a replacement for professional therapy, they can serve as valuable tools in managing the inner critic and fostering a more nurturing internal dialogue. Here are some of the most effective self-help techniques for quietening the voice of the inner critic in the context of C-PTSD:

Mindfulness and Self-Awareness:

Practicing mindfulness can help individuals cultivate self-awareness and observe their thoughts without judgment. By becoming attuned to the inner critic's voice, individuals can begin to recognize its patterns and triggers, allowing them to respond with greater self-compassion and understanding.

Externalizing the Inner Critic:

Externalizing the inner critic involves personifying the critical voice as a separate entity or character. This technique allows individuals to create distance between themselves and the critical thoughts, making it easier to challenge and disarm them.

Challenging Negative Thoughts:

Actively challenging negative thoughts and beliefs perpetuated by the inner critic is essential in quietening its voice. Encourage yourself to question the validity of critical statements and replace them with more balanced and compassionate perspectives.

Self-Compassion Practices:

Cultivating self-compassion involves treating oneself with kindness, understanding, and acceptance, particularly in moments of self-doubt and criticism. Engage in self-compassion exercises such as writing yourself a compassionate letter, practicing loving-kindness meditation, or using self-soothing techniques to comfort yourself.

Reframing Core Beliefs:

Identify and reframe core beliefs instilled by the inner critic that contribute to feelings of unworthiness and inadequacy. Challenge these beliefs by gathering evidence to the contrary and adopting more realistic and empowering perspectives.

Setting Boundaries:

Establishing boundaries with the inner critic involves assertively rejecting its destructive messages and refusing to engage in self-destructive behaviors. Practice setting firm boundaries with the inner critic, reminding yourself that you deserve respect and kindness from yourself.

Gratitude and Appreciation:

Cultivating gratitude and appreciation for oneself can counteract the negative effects of the inner critic. Make a habit of acknowledging your strengths, achievements, and positive qualities, celebrating even small victories and moments of self-care.

Incorporating these self-help techniques into your daily life can empower you to quieten the voice of the inner critic and cultivate a more compassionate and nurturing relationship with yourself. Remember to be patient and gentle with yourself as you navigate this process of healing and self-discovery.

Angering

Angering, as a therapeutic technique, is a powerful tool in the arsenal of strategies for quietening the inner critic within the context of Complex Post-Traumatic Stress Disorder (C-PTSD). Originating from the work of pioneering psychologist Dr. John Bradshaw, anguishing involves harnessing the energy of anger as a means of confronting and disarming the inner

critic's relentless barrage of self-critical thoughts and beliefs.

In the context of C-PTSD, individuals often grapple with deeply ingrained feelings of unworthiness, shame, and self-blame, perpetuated by the critical inner voice derived from past trauma and adverse experiences. The inner critic can wield immense power, undermining self-esteem, eroding self-confidence, and hindering the process of healing and recovery. Angering provides a proactive and assertive approach to challenging the inner critic's dominance and reclaiming one's sense of agency and self-worth.

At its core, angering involves harnessing the energy of anger as a catalyst for change and transformation. Rather than suppressing or denying feelings of anger, individuals are encouraged to embrace and channel this powerful emotion in constructive ways. Anger serves as a potent force for breaking through the barriers of self-doubt and self-criticism, empowering individuals to confront the negative messages of the inner critic head-on.

The process of angering begins with acknowledging and validating the validity of one's anger in response to the injustices and wounds inflicted by the inner critic. Individuals are encouraged to tap into the raw intensity of their anger, allowing themselves to fully experience and express this emotion without judgment or inhibition. This may involve venting anger through journaling, verbal expression, or physical outlets such as exercise or artistic expression.

As individuals embrace their anger, they can begin to direct this energy towards the source of their inner critic's power – the deeply ingrained beliefs and narratives that fuel feelings of shame and inadequacy. Angering involves confronting and challenging these core beliefs, dismantling the distorted perceptions of self-worth and reclaiming one's inherent value and dignity.

Through the process of angering, individuals can develop a sense of righteous indignation towards the inner critic's unjust accusations and condemnations. Anger becomes a driving force for self-advocacy and self-protection, empowering individuals to set firm boundaries, assert their needs, and assertively reject the toxic messages of the inner critic.

However, it's essential to approach angering with mindfulness and intentionality, recognizing that anger is a potent emotion that can be both destructive and transformative. Individuals are encouraged to cultivate self-awareness and emotional regulation skills to navigate angering safely and effectively, avoiding harmful behaviors or expressions that may perpetuate cycles of violence or harm.

In conclusion, angering represents a dynamic and empowering technique for quietening the inner critic in the context of C-PTSD. By harnessing the energy of anger, individuals can confront the negative messages of the inner critic, challenge deeply ingrained beliefs of unworthiness and shame, and reclaim their sense of self-worth and dignity. Through the transformative power of anger, individuals can pave the way towards healing, self-empowerment, and authentic self-expression.

Module 3.2: The Outer Critic

EGGSHELLS OF THE MIND

What is the Outer Critic?

The development of outer critics emerges as a significant psychological phenomenon intricately intertwined with attachment theory. C-PTSD, stemming from prolonged exposure to trauma, particularly in interpersonal relationships or environments where escape is challenging, sets the stage for the emergence of critical external voices that mirror the internalized messages of the inner critic.

These internalized representations influence an individual's beliefs about their worthiness of love, care, and validation, as well as their expectations of others' availability and responsiveness in times of need. In the context of C-PTSD, disruptions in attachment bonds due to neglect, abuse, or inconsistent caregiving contribute to the development of outer critics.

The outer critic can be understood as an externalized manifestation of the critical or abusive dynamics experienced in early attachment relationships. Individuals with C-PTSD may project their internalized self-criticism onto others, particularly authority figures or intimate partners, perpetuating cycles of criticism, blame, and invalidation in their external relationships. These outer critics may mirror the behavior of caregivers or authority figures who were themselves critical, controlling, or emotionally unavailable during childhood.

Moreover, attachment-related schemas and working models of self and others influence individuals' expectations and interpretations of interpersonal interactions. For example, individuals with insecure attachment styles,

such as anxious-preoccupied or dismissive-avoidant, may be particularly prone to developing outer critics as a means of coping with feelings of vulnerability or rejection. Anxious-preoccupied individuals may seek external validation and reassurance from others, while dismissive-avoidant individuals may distance themselves from intimate relationships to avoid perceived threats to their autonomy or independence.

The interplay between attachment-related dynamics and the development of outer critics underscores the complex nature of interpersonal relationships in the context of C-PTSD. Healing from C-PTSD involves not only addressing the overt symptoms of trauma but also unraveling the layers of internalized shame, self-blame, and self-doubt that drive both inner and outer criticism. By fostering self-awareness, self-compassion, and healthier relational patterns, individuals can begin to challenge and transform the critical dynamics that permeate their internal and external worlds, paving the way towards healing, authenticity, and genuine connection with others.

The Outer Critic and Social Anxiety

Individuals with C-PTSD may project their internalized self-criticism onto others, particularly authority figures, peers, or intimate partners, perpetuating cycles of criticism, blame, and invalidation in their external relationships. These outer critics may mirror the behavior of caregivers or authority figures who were themselves critical, controlling, or emotionally unavailable during childhood.

The development of outer critics within the framework of C-PTSD is deeply intertwined with social anxiety and social phobia, two common co-occurring conditions characterized by fear or anxiety in social situations. Individuals with C-PTSD may develop social anxiety or social phobia as a result of traumatic experiences or insecure attachment styles, further exacerbating the critical dynamics present in their external relationships. The fear of judgment, rejection, or humiliation in social settings may intensify the need for external validation and reinforcement of self-worth, leading individuals to adopt critical or defensive behaviors as a means of self-protection.

Moreover, the presence of outer critics in the context of C-PTSD can perpetuate social anxiety or social phobia by reinforcing negative beliefs about oneself and others. Individuals may anticipate criticism or rejection from others based on their internalized experiences of invalidation and abandonment, leading to heightened vigilance and avoidance of social interactions. This cycle of fear, avoidance, and self-criticism further reinforces the critical dynamics present in external relationships, perpetuating feelings of isolation, shame, and inadequacy.

The interplay between outer critics, social anxiety, and social phobia underscores the complex and interconnected nature of trauma-related disorders and their impact on interpersonal functioning. Healing from C-PTSD involves not only addressing the overt symptoms of trauma but also unraveling the layers of internalized shame, self-blame, and self-doubt that drive both inner and outer criticism. By fostering self-awareness, self-compassion, and healthier relational patterns, individuals can begin to challenge and transform the critical dynamics that permeate their internal and external worlds, paving the way towards healing, authenticity, and genuine connection with others.

In conclusion, the development of outer critics in the context of C-PTSD is deeply intertwined with social anxiety and social phobia, perpetuating cycles of fear, avoidance, and self-criticism in social interactions. By recognizing the complex interplay between trauma-related disorders and their impact on interpersonal functioning, individuals can begin to address and heal from the critical dynamics that contribute to their outer critics and social anxiety or social phobia. Through self-awareness, self-compassion, and therapeutic interventions, individuals can reclaim their sense of agency and self-worth, fostering healthier relationships and greater social connectedness.

How to Breakdown the Outer Critic

Quietening the voice of the outer critic within the context of Complex Post-Traumatic Stress Disorder (C-PTSD) can be a challenging but essential aspect of the healing journey. The outer critic, often originating from internalized messages of criticism and invalidation from past traumatic experiences, can perpetuate cycles of self-doubt, shame, and inadequacy. However, there are several effective self-help techniques individuals can employ to challenge and quieten the outer critic's influence, fostering self-compassion, self-worth, and resilience. Here are some of the most effective self-help techniques for quietening the voice of the outer critic in the context of C-PTSD:

Challenge Negative Self-Talk:

Actively challenge negative self-talk perpetuated by the outer critic. Practice identifying and questioning the validity of critical thoughts and beliefs, replacing them with more balanced and compassionate perspectives. Use affirmations or positive self-talk to counteract negative messages.

Reframe Core Beliefs:

Identify and reframe core beliefs instilled by the outer critic that contribute to feelings of unworthiness and inadequacy. Challenge these beliefs by gathering evidence to the contrary and adopting more realistic and empowering perspectives about yourself and your abilities.

Assertiveness Training:

Develop assertiveness skills to assert your needs, boundaries, and rights in relationships and interactions. Practice expressing yourself assertively, advocating for yourself, and setting boundaries with others to protect your emotional well-being and self-esteem.

Inner Child Work:

Engage in inner child work to connect with and nurture the wounded inner child within yourself. Offer the inner child love, validation, and reassurance, addressing their unmet emotional needs and soothing their fears and insecurities.

Mindfulness and Self-Awareness:

Practice mindfulness techniques to cultivate self-awareness and observe the thoughts and emotions triggered by the outer critic without judgment. Mindfulness allows individuals to develop a non-reactive stance towards critical thoughts, fostering greater emotional regulation and resilience. It is from this vantage point of awareness that you can choose to ignore or challenge the nonsense of the outer critic.

END OF MODULE 3

Module 4.1: Toxic Shame

SOUL POISON

What is the Toxic Shame

Toxic shame, a deeply ingrained and pervasive sense of being fundamentally flawed or unworthy, often develops within the complex framework of Complex Post-Traumatic Stress Disorder (C-PTSD). C-PTSD, stemming from prolonged exposure to trauma, particularly in interpersonal relationships or environments where escape is challenging, creates fertile ground for the emergence of toxic shame due to the profound disruptions in attachment bonds and the erosion of a sense of safety, security, and self-worth.

The development of toxic shame within the context of C-PTSD is multifaceted and can be influenced by various factors, including early relational experiences, adverse childhood experiences, and internalized messages of criticism and invalidation. Individuals with C-PTSD may experience shame as a result of neglect, abuse, or abandonment in childhood, leading to the internalization of beliefs that they are inherently unworthy, unlovable, or deserving of mistreatment.

Attachment theory provides valuable insights into how early relational experiences shape the formation of internal working models of self and others, influencing an individual's beliefs about their worthiness of love, care, and validation. Disruptions in attachment bonds due to trauma or dysfunctional caregiving can lead to the internalization of shame-based beliefs and the formation of toxic shame.

For example, children growing up in environments marked by trauma may internalize messages of unworthiness, shame,

and inadequacy from caregivers who are emotionally unavailable, critical, or abusive. These experiences of rejection, invalidation, or betrayal can create deep wounds in the psyche, leading to the development of toxic shame as a means of coping with overwhelming feelings of vulnerability, helplessness, and abandonment.

Moreover, the symptoms of C-PTSD, including emotional dysregulation, dissociation, and interpersonal difficulties, can further exacerbate feelings of shame and self-blame. Individuals with C-PTSD may experience intense shame in response to perceived failures or shortcomings, as well as a pervasive sense of being defective or damaged as a result of their traumatic experiences.

The cycle of shame in C-PTSD is self-perpetuating and insidious, as shame leads to behaviors and coping mechanisms that reinforce feelings of unworthiness and inadequacy. For example, individuals may engage in self-sabotaging behaviors, such as substance abuse, self-harm, or avoidance, as a way of numbing or escaping from painful feelings of shame. These behaviors, in turn, further erode self-esteem and perpetuate the cycle of shame and self-destructive patterns.
Addressing and healing from toxic shame within the context of C-PTSD requires a multifaceted approach that integrates self-awareness, self-compassion, and therapeutic interventions. By recognizing the origins and dynamics of toxic shame, individuals can begin to challenge and reframe shame-based beliefs, cultivate self-compassion and self-worth, and develop healthier coping strategies for managing shame triggers. Therapy modalities such as cognitive-behavioral therapy (CBT), dialectical behavior therapy (DBT), and inner child work can provide valuable tools and techniques for addressing shame and fostering healing and resilience in the aftermath of trauma. Through self-awareness, self-compassion, and therapeutic support, individuals with C-PTSD can begin to break free from the grip of toxic shame and reclaim their sense of worth, dignity, and belonging.

Signs of Toxic Shame

Recognizing the signs and symptoms of toxic shame is crucial for understanding its impact on individuals' well-being and guiding interventions for healing and recovery:

Low Self-Esteem: Individuals experiencing toxic shame often struggle with feelings of low self-worth and inadequacy. They may perceive themselves as fundamentally flawed or unworthy of love, respect, or validation, leading to persistent feelings of self-doubt and self-criticism.

Perfectionism: A common manifestation of toxic shame is perfectionism, the relentless pursuit of flawlessness and validation from others. Individuals may set impossibly high standards for themselves and fear failure or criticism, driving them to overwork or overachieve in an attempt to prove their worthiness.

Self-Blame: Toxic shame often leads to excessive self-blame and self-criticism, even for events or circumstances beyond one's control. Individuals may internalize responsibility for past traumas or adverse experiences, viewing themselves as inherently deserving of mistreatment or punishment.

Difficulty with Boundaries: Individuals with toxic shame may struggle to establish and maintain healthy boundaries in relationships. They may have difficulty asserting their needs or saying no to others, fearing rejection or abandonment if they assert themselves.

Avoidance and Isolation: Shame can lead to a pervasive fear of judgment or rejection from others, causing individuals to withdraw or isolate themselves as a means of self-protection. They may avoid social interactions or situations that trigger feelings of shame or inadequacy.

Self-Sabotage: Toxic shame often fuels self-sabotaging behaviors aimed at numbing or escaping from painful feelings of unworthiness or inadequacy. Individuals may engage in substance abuse, self-harm, or other destructive behaviors as a way of coping with shame and self-loathing.

Difficulty with Intimacy: Shame can hinder individuals' ability to form and maintain intimate relationships. They may struggle to trust others or fear vulnerability, leading to emotional distance or difficulty in expressing their true selves in relationships.

Negative Self-Talk: Inner dialogue characterized by self-critical and demeaning

thoughts is common among individuals experiencing toxic shame. They may berate themselves internally, reinforcing negative beliefs about their worthiness and competence.

Hypervigilance: Constant vigilance for signs of rejection or criticism from others is a hallmark of toxic shame. Individuals may be hyper-aware of others' perceptions of them and constantly seek reassurance or validation to alleviate feelings of insecurity.

Psychosomatic Symptoms: Toxic shame can manifest somatically, contributing to physical symptoms such as headaches, gastrointestinal issues, or chronic pain. The mind-body connection means that unresolved emotional distress can manifest as physical symptoms.

How to Reduce Toxic Shame

Reducing symptoms of toxic shame within the context of Complex Post-Traumatic Stress Disorder (C-PTSD) requires a multifaceted approach that integrates evidence-based self-help strategies aimed at challenging shame-based beliefs, fostering self-compassion, and promoting resilience. While therapy with a qualified mental health professional remains an essential component of healing from C-PTSD, individuals can also utilize self-help techniques to address and alleviate symptoms of toxic shame. Here are some of the most effective evidence-based self-help strategies for reducing symptoms of toxic shame in the context of C-PTSD:

Education and Self-Help Resources: Educating oneself about toxic shame, trauma, and self-help techniques can empower individuals to take an active role in their healing journey. Reading books, attending workshops, or accessing online resources about C-PTSD and shame can provide valuable insights and tools for reducing symptoms and fostering resilience.

Behavioral Activation: Engaging in activities that bring pleasure or fulfillment can counteract feelings of worthlessness and disconnection associated with toxic shame. Individuals can create a list of enjoyable activities and schedule them regularly to promote feelings of joy and self-worth.

Cognitive Restructuring: Cognitive restructuring involves identifying and challenging distorted or irrational thoughts and beliefs associated with toxic shame. Individuals can use techniques such as cognitive-behavioral therapy (CBT) worksheets to examine the evidence supporting or refuting shame-based beliefs, replacing them with more realistic and balanced perspectives.

Self-Compassion Practices: Self-compassion involves treating oneself with kindness, understanding, and acceptance, particularly in moments of distress or self-criticism. Techniques such as self-compassion meditations, loving-kindness meditation, and writing oneself compassionate letters can help individuals cultivate self-compassion and counteract the effects of toxic shame.

Targeting the Inner/Outer Critics:
Using the techniques outlined in the previous module, effective management and reduction of the inner and outer critics will lead to significantly less internal shame-based messaging.

Incorporating these evidence-based self-help strategies into one's daily life can empower individuals to reduce symptoms of toxic shame and cultivate greater self-compassion, self-worth, and resilience in the context of C-PTSD. It's important to approach these techniques with patience, consistency, and self-compassion, recognizing that healing from toxic shame is a gradual and ongoing process that requires dedication and self-care.

END OF MODULE 4

CONGRATULATIONS!

You have made to the end of the Self-Therapy Guidebook for Complex PTSD! This is no small feat, as challenging our habits, some of which have been lifelong, is an arduous and exhausting process, but one that is ultimately rewarding and inspiring. While this guidebook was intended to be read from start to finish as a simulation of a course of therapy administered by a professional, it is highly recommended that you return to any section of the content as needed to maintain upkeep of your emerging recovery skillset.

As a final note, any process of personal change is hard, and requires reserves of willpower, disciplined action, and a clear goal or 'why' for undertaking it. Mental health, like anything else, is largely a composite of healthy micro-behaviours conspiring to improve your overall quality of life, so take it slow, and steadily build the behaviours necessary to facilitate this process.

Once again, congratulations for taking steps forward to improve your life, and we wish you the very best on the road to recovery from Complex PTSD.

The Greylight Psychology team